

Dermatologist, Spectrum Dermatology
402-223 Nelson's Cres
New Westminster, BC V3L 0E4

Apr 13, 2026

RE:
DOB:
PHN:
Phone:Home
Personal
Addr:

Date Seen:13-Apr-2026

Dear

Progress Note

Interval history	Efudex: face and nose bid x 3 weeks Noted redness, crusting to the tx sites Was able to tolerate treatment
PMH	HTN, Dyslipidemia, knee replacement
Medications	Ramipril, Atorvastatin, HCTZ

13-Apr-2026

Actinic keratoses- s/p Efudex, 50% improvement

Persistent ill-defined rough erythematous papules to the nose cheeks and forehead

1. Cryo applied today
2. Discussed Efudex 1 week bid per month, defer until after summer
3. Should any treated or other AK lesions become thick, indurated or tender, recommend re-referral for possible SCC since there is low risk of transformation.
4. Follow up: 6 weeks with LPN for cryo until end of summer then Efudex maintenance therapy

- discussed vascular laser for nasal telangiectases and erythema to cheeks/nose

Left nose actinic keratosis, FRS26-2233

1. Clear today, nfar

09-Jan-2026

Diagnosis and Plan:

Rule out squamous cell carcinoma to left nose

2-3mm flesh coloured discrete papule to left nasal ala. Notes recurrent bleeding

1. Excisional biopsy to the above site under local anesthesia was performed today with verbal consent. Risks of bleeding, infection, recurrence and scar were reviewed. Wound care with Fucidin ointment and bandage. Post procedure care reviewed with patient. Specimen sent for histopathology.

PHN:
DOB:
Gender:
Provider:

Chart#:
Age:

2. She is prescribed Efudex cream for use as a field treatment twice daily for 4 weeks which she will start in Feb after her trip to Mexico. Expected treatment effect, i.e. erythema, crusting and skin tenderness were reviewed in detail. The patient is also advised to limit sun exposure, as Efudex can make the skin more sensitive to UV rays.
3. Follow up with LPN: phone call for results

Actinic keratoses

Ill-defined rough erythematous papules to the face and left nasal sidewall.

1. Counselling on etiology and prognosis of AKs. I advised regular sunscreen to prevent recurrence and non-melanoma skin cancer. Single cycle cryotherapy was applied today; we reviewed healing course including potential for blistering.
2. Should any treated or other AK lesions become thick, indurated or tender, recommend re-referral for possible SCC since there is low risk of transformation.

Return in Apr 2026 - assess for Efudex response and skin review; sooner management prn according to bx results

COMMENT:

Location: left nasal sidewall

Duration: months

Painful: Y; Bleeding: N; Changing: Y, grown

Previous skin cancer (basal/squamous cell carcinoma, melanoma)? Y. Location/date: BCC x2 right nose SCC to right shin

History of melanoma in parent/siblings?: Unknown

Have you ever used an indoor tanning bed?: N

How many years have you spent in a job where you work mainly outdoors in the sun?: 0

Have you ever had radiation therapy?: N

Regarding results

Patients are contacted directly with all test results. Referring physicians and nurse practitioners are notified only of abnormal or clinically significant findings (e.g., biopsies with pathology, laboratory abnormalities, or positive margins) that require follow-up or coordination of care. Normal results are communicated to patients but are not routinely sent to referring providers.

Yours sincerely,